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Ministry of Public Health, Hygiene and Social Welfare
National Institute of Public Health
Public Health Emergency Operations Center
PRESIDENTIAL EBOLA TASK FORCE 17
MVE17 Incident Management System



Situation Report of the 17th Ebola Virus Disease Outbreak / DRC

SitRep N°178/MVE-BDBV/08/11/2026

Reporting date: November 8, 2026 Publication date: November 9, 2026

HIGHLIGHTS (last 24 hours)

Over the past 24 hours, seventy (70) new confirmed cases and 31 confirmed deaths, including 22 community deaths and 9 deaths within treatment centers, have been reported. The new cases were recorded in Ituri (40), North Kivu (23), Tshopo (2), and Haut-Uélé (5). A total of 614 new samples were analyzed in
The laboratories that submitted their data; 70 of these samples tested positive for the Bundibugyo virus, representing a positivity rate of 11.4%. No new health zones have been affected in the last 24 hours.

ACCUMULATIVE CASE	ACCUMULATIVE DEATH CONFIRMED	LETHALITY	PATIENTS IN ISOLATION/CTE	ACCUMULATIVE HEALED	TRACKING RATES CONTACTS (Of the Day)
10990	5354	48.7%	966	2951	79.2%

Affected provinces and affected health zones

8 Provinces affected

Haut-Uélé, Ituri, North Kivu, South Kivu, Tshopo, Bas-Uélé, South Ubangi, and North Ubangi

67 out of 178 health zones affected (37.6%)

Summary of key points from the last 24 hours

On November 8, 2026, seventy (70) new confirmed cases of Ebola virus disease (EVD) were reported in four provinces: Ituri (40 cases), North Kivu (23 cases), Haut-Uélé (5 cases), and Tshopo (2 cases), representing a 70.7% increase compared to the previous day. Among these cases, 22 community deaths were reported (15 in Ituri, 6 in North Kivu, and 1 in Tshopo). In addition, 9 confirmed deaths were recorded in Ebola Treatment Centers (ETCs).
(4 in Ituri, 4 in North Kivu and 1 in Haut-Uélé), bringing the total number of confirmed deaths reported during the period to 31.

Furthermore, 11 patients (1 in North Kivu, 2 in Bas Uele, 2 in Tshopo and 6 in Ituri) were declared cured after obtaining two negative results in the control test for Ebola Virus Disease.

Since the start of the epidemic, the DRC has recorded 10,990 confirmed cases, including 5,354 deaths, representing an overall case fatality rate of 48.7%. Ituri remains the epicenter of the epidemic, accounting for 69.6% of cumulative confirmed cases and 57.1% of new reported cases, while North Kivu represents 32.9% of new cases and has a case fatality rate of 54.7%, higher than the national average.

No new health zones have been affected in the last 24 hours.

Thus, 67 out of 178 health zones (37.6%), spread across eight provinces, have been affected since the start of the epidemic, namely: Ituri, North Kivu, Haut-Uélé, Tshopo, South Kivu, Bas-Uélé, South Ubangi and North Ubangi.

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Table 1. Distribution of confirmed cases and deaths by affected province, as of November 8, 2026.

Province	New confirmed cases (24h)	Confirmed cases	Deaths (confirmed)	Lethality	Affected health zones
Ituri	40	7 654	3,613	47.2%	28/36 (77.8%)
North Kivu	23	2 720		54.7%	17/34 (50.0%)
Haut-Uélé	5	491		43.6%	7/13 (53.8%)
Tshopo	2	98	1,488,214 32	32.7%	7/23 (30.4%)
South Kivu	0		1	33.3%	1/34 (2.9%)
Bas-Uélé	0	3	4	30.8%	4/11 (36.4%)
South Ubangi	0		1	50.0%	1/16 (6.3%)
North Ubangi	0	13 2 9	1	11.1%	2/11 (18.2%)
Total	**70**	**10,990**	**5,354**	**48.7%**	**67/178 (37.6%)**

Table 2. Distribution of confirmed cases and deaths by province and zone as of November 8, 2026

Province / Zone of rate	Cumulative number			Current situation (24 hours)			
	Cases (n)	Deaths (n)	Health fatality	New. Case	Death Confirmed community (Swab+)	Death deaths intra-CTE	confirmed
Ituri	**7654**	**3613**	**47.2% 40** 9.1%		**15 4 19**		
Adja	11	1	28.0%				0
Aru	25	7	25.0%				0
Ariwara	8	2	41.7%				0
Aungba		5	22.7%				0
Bamboo		50	100.0%				0
Boga	12,220,2	2	35.2%				0
Bunia	2047	721	24.4%	6	3	1	4
Damascus	41	10	50.0%				0
Drodro	16	8	36.5%	1	1	0	1
Fataki	85	31	63.6%				0
Gethy	11	7	0.0%				0
Kambala	2	0	34.1%				0
Kilo	41	14	61.7%	1	1	0	1
Komanda 193 339			49.9%				0
Lita		119,169	71.4%	1	0	0	0
Logo	14	10	29.7%				0
Lolwa	37	11	16.7%				0
Mahagi	6		53.3%				0
Mambasa 135		1 72	52.5%	3	3	1	4
Mandima 570 419			20 42.2% 4			0	7
Mangala			49.2% 1 58.3%			0	0
Mongbwalu 732		299,177,360	36.8%		700	0	0
Nia-Nia 345 867					0	0	0
Nizi		201,319		1 1	0	1	1

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Nyankunde 141		45	31.9%				0
Rimba 29			24.1%				0
Rwampara 1220			36.5%	1	0	1	1
Tchomia 86		7,445,27	31.4%				0
To ventilate 0		493	N / A				0
North Kivu 2720		**1488**	**54.7% 23**		**6**	**4**	**10**
Blessed	823	528	64.2%	8	3	0	3
Biena 27		11	40.7%				0
Butembo 351		204	58.1% 2		0	0	0
Goma	1		0.0%				0
Kalunguta 111		0	44.1%	1	0	2	2
Katwa 685		49	61.8%	1	0	0	0
Kayna	3	423 2	66.7%				0
Kyondo 47		22	46.8%				0
Lubero	45	16	35.6% 2		0	0	0
Mabalako 127		41	32.3%				0
Alimbongo 6		4	66.7%				0
Manguredjipa 16		5	31.3%				0
Masereka 48		15	31.3%				0
Musienene 164		81	49.4%				0
Mutwanga 101		24	23.8%	6	2	0	2
Oicha 106		42	39.6% 2		0	2	2
Vuhovi 59		21	35.6%	1	1	0	1
Haut-Uélé 491		**214**	**43.6%**	**5**	**0**	**1**	**1**
Boma Mangbetu	48	20	41.7% 2		0	0	0
Dungu	1	1	100.0%				0
Gombari	3	1	33.3%				0
Isiro	123	58	47.2% 2		0	1	1
Pawa	126	68	54.0%				0
Rungu	9	5	55.6%				0
Wamba	181	61	33.7%	1	0	0	0
Tshopo	**98**	**32**	**32.7%** 2		1	0	1
Bafwasende 18		3	16.7%				0
Kabondo	24	5	20.8%				0
Lubunga	1	0	0.0%				0
Makiso - Kisangani	38	15	39.5%	1	0	0	0
Mangobo 11			72.7%	1	1	0	1
Tshopo	5	80	0.0%				0
Wanie - Rukula	1	1	100.0%				0
South Kivu 3		**1**	**33.3%**	0	0	0	0
Miti-Murhesa 3		1	33.3%				0
Bas Uélé	**13**	**4**	**30.8%**	0	0	0	0
Buta	1	1	100.0%				0
Viadana	6	1	16.7%				0
Dakwa	1	0	0.0%				0
Ganga	5	2	40.0%				0
South Ubangi 2		**1**	**50.0%**	0	0	0	0
Bulu	2	1	50.0%				0

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North Ubangi 9	**1**	**11.1%**	0	0	0	0
Wapinda 1 5 Yakoma 4 0 **Total 10990**		20.0%				0
5354 *These are the confirmed deaths		0.0%				0
that occurred in the various CTEs of the		**48.7%**	**70**	**22**	**9**	**31**

province which are being broken down to be classified by health zone.
NA: Not Applicable.

Table 3. Status of alerts notified by province for November 8, 2026

DPS	Alerts Received		Total alerts	Alerts verified and validated (suspected case)		Alerts verified and invalidated (no suspected cases)		Number of suspects investigated	Number of suspects investigated and transferred At CT/CTE (of the day)
	Living	Deceased		Living	Deceased	Living	Deceased		
Haut-Uélé	46	2	48	11	2	35	0	13	3
Bas Uélé	82	1	83	3	1	79	0	4	0
Ituri	1 098	79 1	1177	110	73	981	0	181	97
North Kivu	771	69 14 0	1840	145 14	68	1626		213	87
South Ubangi			36	3	0		1		0
North Ubangi	36 0 41 3		44	13	0	11	0	3	6
Tshopo				9	3	23 32	0 0	13 12	3
Total General	**3088**	**154 3242 294**			**147,2787**		**1**	**439**	**196**

Figure 1. Evolution of cases by date of symptom onset over the last 21 days

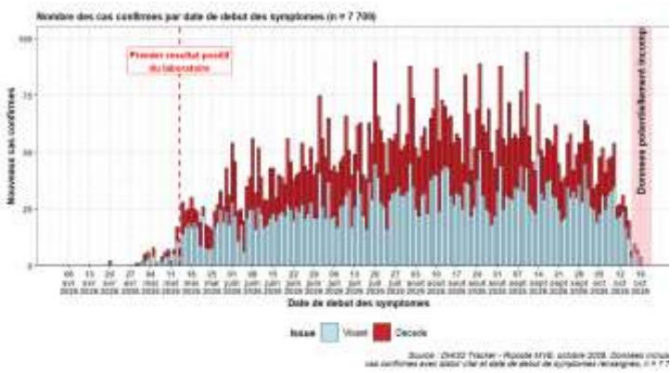
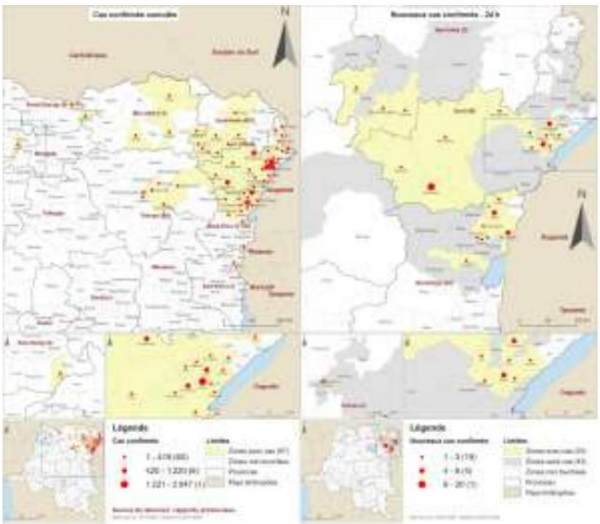


Figure 2. Spatial distribution of confirmed cases (24 hours and cumulative) by health zone.



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1. COORDINATION

1.1. Main Actions

• Support for response activities in the provinces **2.**
RESPONSE ACTIONS BY PILLAR

2.1. Epidemiological surveillance

2.1.2. Main Actions

As of November 8, 2026, 3,242 alerts had been reported in the seven provinces that submitted their data (Bas-Uélé, Haut-Uélé, Ituri, Nord-Kivu, Sud-Ubangi, Nord-Ubangi, and Tshopo), including 3,088 alerts of living persons and 154 of deaths; 3,229 were verified (99.6%). Of these, 441 (13.7%) were validated as suspected cases and 2,788 (86.3%) were invalidated. Of the 439 suspected cases investigated, 196 were transferred to COVID-19 treatment centers (Table 3).

As of November 8, 2026, a total of 38,453 contacts were under follow-up in the six provinces that reported contacts to follow up, of which 30,444 were seen in the last 24 hours, representing a follow-up rate of 79.2%, below the required threshold of 95%.

Table 4. Status of contact tracing by province, as of November 8, 2026

Province	Contacts under follow-up	Contacts viewed	tracking rate
Ituri	16,447	13,738	83.5%
Haut-Uélé	1,370	1,145	83.6%
North Kivu	19,029	14,583	76.6%
Tshopo	1228	612	49.8%
Bas-Uélé	267	263	98.5%
South Ubangi	112	103	92.0%
North Ubangi	0	0	N / A
Total	38,453	30,444	79.2%

2.2. Update on checkpoint and point of entry (PoE/PoC) activities

2.2.1. Main actions

• **North Kivu:** On November 8, 2026, out of 86,150 travelers passing through entry and control points, 84,047 were screened and washed their hands (97.56%), while 2,103 refused to wash their hands, representing a refusal rate of 2.44%. Among the 21 strategic sites, 20 declared their PoE/PoC (95.2%), and 2 alerts were notified and subsequently validated after thorough investigation. The Butuhe PoC was not operational following the security incident of September 26, 2026.

• **Ituri :** 139,982 travelers, 97.2% screened, 96.7% handwashing. 90.3% completeness (56/62). 2 alerts, including 1 intercepted body

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- **Tshopo :** 13,295 travelers, 99.4% screened, 0 alerts. Completeness 55.6% (10/18). Several ports Private businesses are closed on Sundays because of taxes.
- **Haut-Uele :** 17,293 travelers on 10/13 PoCs, 98% screened, 279 refused Screening, 0 alerts.

Challenges :

Insecurity on the Bolebole road, unpaid wages, lack of ambulances, and pressure from state services at PK6 who let travelers through without screening

Table 5. Status of key PoE/PoC indicators by province, as of November 8, 2026

Indicators	Ituri	North-Kivu	South Kivu	Tshopo	Bas-Uele	Overall
Report completeness	92% (57/62)	95% (20/21)	56% (5/9)	100% (18/18)	50% (11/22)	84.1% (111/132)
Strategic PoC/PoE	1	N / A	N / A	N / A	N / A	1
Number of past travelers	147 103 95	793 7 173		23,989	2,345	276,403
Proportion of travelers screened	97%	99%	98%	100%	98%	98%
% of travelers who washed their hands	96%	99%	98%	99%	98%	97%
% of those travelers made aware	100%	99%	100%	100%	76%	99%
Number of alerts notified	02	02	00	00	00	4
Number of validated live alerts	02	01	00	00	00	3
Number of lifeless bodies intercepted	00	01	00	00	00	1
% of suspected cases transferred to CT/CTE	100% (2/2)	100% (1/1)	0%	0%	0%	100% (3/3)
% of lifeless bodies swabed/secured	0%	100% (1/1)	0%	0%	0%	100% (1/1)
% of suspected cases investigated within 24 hours	100% (2/2)	100% (2/2)	0%	0%	0%	100% (4/4)

Laboratory

2.2.2. Main actions

- **Ituri:** 40 new positive results (14 living and 10 dead) out of 279 samples received and analyzed (positivity: 8.6%).

North **Kivu:** 23 new confirmed cases (17 living and 6 dead); laboratories in Beni and Butembo have 279 samples were received and 276 were analyzed (positivity: 4.3%)

- **Tshopo :** 11 samples taken (8 living, 3 deceased), 2 new cases confirmed. Challenges: impending diesel shortage (incinerator and group), reagent break (MVE buffer) and persistent invalid SWAB.
- **Haut-Uele :** 30 samples, 5 positive out of 13 new samples and 5 out of 17 repeat samples. Positivity rate for the day 16.7%, overall 19.8%.

2.3. Infection Prevention and Control (IPC/EDS)

2.3.1. Main actions

- **Ituri :** 11/12 health facilities and 80/82 households decontaminated, but 19 out of 35 rings remain open. Shortage teams in Mangala and Mandima, and inputs in Mandima and Mambasa.
- **North Kivu :** 18/27 rings opened (66.7%), 17/20 ESS and 13/21 households decontaminated, 0 PPL Infected in 12 cases. None of the decontaminated households received a kit. Average PCI score of 46.9%.

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- **Tshopo :** only 8/110 ESS exceed 80% of the PCI score, 49/110 have a triage circuit and 59/110 received kits. 100% of affected social and solidarity economy (SSE) facilities have been decontaminated (90/90), and 89.6% of households. 9/25 infected service providers have been investigated.
- **Haut-Uele :** 1/1 ring open and 7/7 households decontaminated, 0 ESS decontaminated. Lack of PCI kits. Supervisors in Rungu, Dungu, Boma Mangbetu and Gombari are not active.

Dignified and safe burials (DSH)

- **Ituri :** 64/77 EDS carried out (83%), 59/68 bodies swabed, 4 failures, 9 postponements.
- **North Kivu :** 54 EDS and 56 bodies recovered for 68 alerts, 28 reports. One failure in Musienene (community resistance). This data is from 05/11.
- **Tshopo :** out of 14 alerts, only 1 swabé body and 1 EDS, 13 failures (non-execution of the order), 5 reports.
- **Haut-Uele :** 8 alerts, 8 swabs, 8 EDS. The EDS team trained by FHI360 is not activated in Isiro.

2.4. Holistic care

2.4.1. Main actions

- **Ituri :** 347 patients in beds (127 confirmed, 220 suspected), 56 admissions, 70 discharges (6 recovered, 12 deaths). Bed occupancy 34%. Hot meals for 96% of patients, but 15 patients from Mandima were not fed.

  Challenge: Bloodletting. Strike by CTE Kasenyi staff (non-payment).

- **North Kivu :** 535 patients in beds (184 confirmed, 351 suspected, including 74 children under 5 years old), 90 Admissions: 109 discharges (1 recovered, 100 non-cases, 8 deaths). 11 critically ill patients (Katwa 8, Oicha 3). 986 hot meals served. 155 patients are hospitalized outside of the CTE/CT.

- **Tshopo :** 6 patients (2 confirmed, 4 suspected) at the CTE Météo, occupancy rate of 18.8%. 2 recovered and discharged. 679 people received the experimental treatment (including 67 confirmed cases) and 612 people received post-exposure prophylaxis.

- **Haut-Uele :** 21 confirmed cases and 34 suspected cases hospitalized, 1 death of a confirmed case within the last 24 hours in Isiro. 3 Critical cases in Pawa. Boma Mangbetu is overcrowded with suspected cases (137.5%). Challenges: demotivation (non-payment), network, nutritional inputs, water and energy.

2.5. Vaccination

2.5.1. Main actions

North **Kivu :** 2,599 vaccinated in 4 health zones (Beni 75.9%), including 5 people vaccinated in Kanzulinzuli. Stock depleted (2,600 doses received, 2,599 used, 1 lost). The three other provinces have no vaccination activities.

2.6. Continuity of Care

2.6.1. Main actions

- **North Kivu :** evaluation of Manguredjipa hospital. No triage, protective equipment Insufficient, 0% of providers trained in emergency obstetric care, 55 staff exposed and 13 in quarantine.

- **Tshopo :** evaluation of 3 social and solidarity economy (SSE) facilities. No trained service providers, no triage or incinerator, decrease in attendance at curative services, prenatal consultations and vaccinations, and a strike by public employees.

2.7. Risk Communication and Community Engagement (RCCE)

2.7.1. Main actions

- **Ituri :** 29% completeness (8/28 health zones), 303 community alerts, 66,662 people reached through home visits. Challenges: demotivation of stakeholders (Nizi axis) and insufficient reporting forms.

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North **Kivu :** 18,704 home visits (60,201 people) and 534 educational talks. An emergency meeting addressed the ADF massacres in Aloya, which resulted in the loss of contact with suspects and other individuals. Rumors circulated about a healthcare workers' strike, and a complaint was filed in Katwa regarding police harassment.

- **Tshopo :** 77 community alerts (34 resolved). The prevailing rumor is "Ebola is a business", "Ebola does not exist" and "Ebola is political".
- **Haut-Uele :** 10 mass activities in churches, reaching 26,756 people. Planned: partnership with 15 radio stations, motorized caravan, and teacher briefings. Challenges: mobility and computer equipment.

Mental health and psychosocial support (MHPSS)

2.7.2. Main actions

- **Ituri :** 19/24 latest confirmed cases covered, but only 15/28 health zones. 54 results announced (19 positives). No data from Fataki.
- **North Kivu :** 17/24 cases covered (70.8%), 104 results announced (52 positive), 28/36 children monitored in the community. Low coverage in 9 health zones, no nurseries in 10 health zones, lack of logistics.
- **Tshopo :** 100% of suspected and confirmed cases followed, 2 resistances notified (1 lifted, in Bethsaida). Need to train the APS of Wanierukula and provide means of transport.

2.8. Logistics

2.8.1. Main actions

- **North Kivu :** delivery of 41 bottles of gel, 200 Tyvek (PoE from Nobili and Kasindi), 72 pairs of glasses (ECUMERS Beni) and 5 tarpaulins (Byakato).
- **Tshopo :** Construction of the Makiso isolation junction and expansion of the Meteorological Center. 115 motorcycles are needed. for the ZS.

2.9. Security

2.9.1. Main actions

- Insecurity: ADF in Mabalako (44 bodies counted, attempted arson at the Beni CTE), Bafwasende in Tshopo (activities suspended), Bolebole axis. In Haut-Uélé; strikes in all four provinces due to unpaid wages
- Resistance and rumors: refusal to swab and isolate, "Ebola is a business".

2.10. PSEA (Prevention of Sexual Exploitation and Abuse)

2.10.1. Main actions

- **Ituri :** only 15/36 health zones (41.7%) are aware of the reporting mechanism. Lack of support awareness-raising, computer kits and stakeholder mapping.
- **North Kivu :** 5/34 health zones (14.7%) are familiar with the mechanism. 99 staff members have been briefed, all of whom have signed the code of conduct. Awareness-raising sessions have been held in Katwa, Butembo, Beni, Kalunguta, and Mutwanga.
- **Tshopo :** complaints mechanism installed in 11 health areas (7.4% of affected areas) (covered). Nurses' strike, missing tools.

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KEY MESSAGES

Seventy (70) new confirmed cases were reported in four provinces, including 40 in Ituri, 23 in North Kivu, 5 in Haut-Uélé and 2 in the Tshopo. The transmission remains particularly marked in the ZS from Mandima (20 cases) and Bunia (6 cases) in Ituri, as well as in those of Beni (8 cases) and Mutwanga (6 cases) in North Kivu.
Of the 31 confirmed deaths reported, 22 are deaths community-based (15 in Ituri, 6 in North Kivu and 1 in Tshopo) and 9 are deaths at CTEs (4 in Ituri, 4 in North Kivu and 1 in Haut-Uélé). The contact tracing rate is 79.2% (30,444/38 453) in the six provinces with contacts under follow-up (Bas-Uélé: 98.5%; Sud-Ubangi: 92.0%; Haut-Uélé: 83.6%; Ituri : 83.5%; North Kivu: 76.6%; Tshopo: 49.8%), up by compared to the previous day (69.8%) but still below the 95% threshold, with the lowest rate in Tshopo, where activities remain suspended in Bafwasende due to insecurity.

TIMELINE OF KEY EVENTS

From April 24 to May 15, 2026, the epidemic is moving beyond the detection of the first cases suspected of having the virus confirmed by biological testing Ebola Bundibugyo, leading to the declaration official of the 17th epidemic of Virus Disease Ebola in the Democratic Republic of Congo.

Between May 17 and 18, 2026, the authorities national, with the support of partners, have strengthened the response with the declaration of the public health emergency of scope national and then continental, accompanied by

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